**Apollo Imperial HOSPITALS**
(Owned by Imperial Hospital Limited)

# DISCHARGE SUMMARY

**MRN** : IHL1037895
**Patient Name** : M MOHSIN CHOWDHURY
**Age/Gender** : 59Y 1M 30D/Male
**Phone** : 01713048084
**Address** : D7, ABC BAY VIEW ROAD NO# 3, KHULSHI, CHITTAGONG
**Primary Consultant** : DR. MOHAMMAD MAJIBUL HAQUE

**Admission ID** : IA12609032942
**Admission Date** : 08-Sep-2026 12:43 AM
**Bed Number** : 2-DB-33
**Department** : INTERNAL MEDICINE
**Discharge Type** : Discharge With Approval
**Discharge Date** : 10-Sep-2026

## DIAGNOSIS

DUODENAL ULCER WITH MALENA WITH HTN , H/O BARIATRIC SURGERY , DYSLIPDAEMIA , IDIOPATHIC HYPERSOMNIA WITH PARASOMNIA

## HISTORY & EXAMINATION

**Chief Complaint :** H/O PRESYNCOPAL ATTACK 1 HOUR BACK FOLLOWED BY ACCIDENTAL FALL ON THE FLOOR , BLUNT TRAUMA OVER BACK OF THE HEAD DUE TO FALL ON THE FLOOR

**Clinical History :** HE IS ORIENTED AND CONSCIOUS. BP-100/70 MMHG,PULSE-92 BEAT/MIN,SPO2-100 % IN ROOM AIR,RR-20 BREATH/MIN,TEMP- 98.°F , HEART/LUNG-NAD/CLEAR, CBG- 7.3mmol/L, GCS: 15/15, PUPIL:REACTING TO LIGHT:PLANTER:BILATERAL EXTENSOR , MUSCLE POWER- 5/5 IN ALL FOUR LIMBS , DEHYDRATION++, A 59-year-old male presented to the Emergency Department with a history of a presyncopal attack approximately 1 hour prior to presentation, followed by an accidental fall on the floor. He sustained blunt trauma to the occipital region/back of the head during the fall. On examination, the patient was conscious and oriented. GCS was 15/15. Blood pressure was 100/70 mmHg, pulse rate 92 beats/min, respiratory rate 20 breaths/min, $SpO_2$ 100% on room air, and temperature 98°F. CBG was 7.3 mmol/L. Pupils were bilaterally reactive to light. Bilateral plantar responses were extensor. Muscle power was 5/5 in all four limbs. Cardiovascular and respiratory examinations revealed no significant abnormality, with clear lungs. Dehydration was present (++). His past medical history was significant for hypertension, dyslipidaemia, previous bariatric surgery, and idiopathic hypersomnia with parasomnia. He is being admitted under Medicine for further evaluation of presyncope, with ECG advised.

**Past Illness :** HTN , H/O BARIATRIC SURGERY , DYSLIPDAEMIA , IDIOPATHIC HYPERSOMNIA WITH PARASOMNIA

## HOSPITAL COURSE MANAGEMENT

After clinical assessment at ER patient was admitted into IPD under DR. MOHAMMAD MAJIBUL HAQUE [INTERNAL MEDICINE]. Then patient was thoroughly assessed and managed accordingly. Now his clinical condition is improved and he is hemodynamically stable and afebrile. So patient is being discharged with advice.

## PATHOLOGY INVESTIGATION(S) :

| Date | Test Name | Attribute | Result | Normal Range |
|---|---|---|---|---|
| 09-09-2026 | APTT | Patient Value APTT | **28.2** Second | 21.5 - 32.6 |
| | | Control Value APTT | **28.3** Second | |
| 09-09-2026 | PROTHROMBIN TIME (PT & INR) | Index | **105.04** % | |
| | | International Normalized Ratio(INR) | **1.04** | |
| | | Prothrombin Ratio (PR) | **0.95** | |

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| **Primary Consultant** | : DR. MOHAMMAD MAJIBUL HAQUE | **Discharge Date** | : 10-Sep-2026 |

## PATHOLOGY INVESTIGATION(S) :

| Date | Test Name | Attribute | Result | Normal Range |
|---|---|---|---|---|
| | | Patient value (PT) | **11.9** Sec | 10.00 - 14.00 |
| | | Control Value (PT) | **12.5** Sec | |
| 09-09-2026 | Fibrinogen | FIBRINOGEN-PLASMA | **2.2** g/L | 2 - 4 |
| 09-09-2026 | D-Dimer | D-DIMER-PLASMA | **0.13** ug FEU/mL | <0.50 |
| 09-09-2026 | CBC | Basophils# | **0.01** 10^9/L | 0.02-0.1 |
| | | Eosinophils# | **0.19** 10^9/L | 0.02-0.5 |
| | | Monocytes# | **0.38** 10^9/L | 0.2-1.0 |
| | | Lymphocytes# | **2.23** 10^9/L | 1-3 |
| | | Neutrophils# | **5.33** 10^9/L | 2-7 |
| | | Basophils | **0.1** % | 0-2 |
| | | Eosinophils | **2.3** % | 1-6 |
| | | Monocytes | **4.7** % | 2-10 |
| | | Lymphocytes | **27.4** % | 20-40 |
| | | Neutrophils | **65.5** % | 40-80 |
| | | Total Leucocyte Count | **8.13** 10^9/L | 4.0 - 10.0 |
| | | Mean platelet volume (MPV) | **11.0** fL | 6.5-12.0 |
| | | Platelets Count | **200** 10^9/L | 150 - 450 |
| | | RDW-SD | **46.1** fL | 39 - 46 |
| | | RDW-CV | **14.2** % | 11.6 -14.0 |
| | | MCHC | **36.1** g/dL | 27 - 32 |
| | | MCH | **31.8** pg | 27 - 32 |
| | | MCV | **88.0** fL | 83 -101 |
| | | Red Blood Cells (RBC) | **2.74** 10^12/L | 4.5 - 5.5 |
| | | HCT /PCV | **24.1** % | 40.0 - 50.0 |
| | | HB (Haemoglobin) | **8.7** g/dL | 13.0 - 17.0 |
| 09-09-2026 | STOOL OCCULT BLOOD | Occult Blood Test (OBT) | **Positive** | Negative |
| 09-09-2026 | STOOL-ROUTINE AND MICROSCOPY | MUCUS CELLS | **NIL** /HPF | |
| | | MUSCLE FIBRES | **NIL** /HPF | |

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| **Primary Consultant** | : DR. MOHAMMAD MAJIBUL HAQUE | **Discharge Date** | : 10-Sep-2026 |

## PATHOLOGY INVESTIGATION(S) :

| Date | Test Name | Attribute | Result | Normal Range |
|---|---|---|---|---|
| | | VEGETABLES CELLS | **NIL** /HPF | |
| | | FAT GLOBULES | **NIL** /HPF | |
| | | EPITHELIAL CELLS | **NIL** /HPF | |
| | | PUS CELLS | **0-2** /HPF | |
| | | RED BLOOD CELLS | **0-2** /HPF | |
| | | REACTION | **ALKALINE** | |
| | | MUCUS | **NIL** | |
| | | CONSISTANCY | **SEMI SOLID** | |
| | | BLOOD | **NIL** | |
| | | AMOUNT | **SUFFICIENT** | |
| | | ODOUR | **OFFENSIVE** | |
| | | COLOUR | **BLACKISH** | |
| | | STARCH | **NIL** /HPF | |
| | | OVA | **NIL** /HPF | |
| 10-09-2026 | CBC | Basophils# | **0.01** 10^9/L | 0.02-0.1 |
| | | Eosinophils# | **0.2** 10^9/L | 0.02-0.5 |
| | | Monocytes# | **0.38** 10^9/L | 0.2-1.0 |
| | | Lymphocytes# | **1.68** 10^9/L | 1-3 |
| | | Neutrophils# | **5.62** 10^9/L | 2-7 |
| | | Basophils | **0.1** % | 0-2 |
| | | Eosinophils | **2.5** % | 1-6 |
| | | Monocytes | **4.8** % | 2-10 |
| | | Lymphocytes | **21.3** % | 20-40 |
| | | Neutrophils | **71.3** % | 40-80 |
| | | Total Leucocyte Count | **7.88** 10^9/L | 4.0 - 10.0 |

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| Date | Test Name | Attribute | Result | Normal Range |
|---|---|---|---|---|
| | | Mean platelet volume (MPV) | **10.6** fL | 6.5-12.0 |
| | | Platelets Count | **220** 10^9/L | 150 - 450 |
| | | RDW-SD | **45.0** fL | 39 - 46 |
| | | RDW-CV | **14.3** % | 11.6 -14.0 |
| | | MCHC | **35.5** g/dL | 27 - 32 |
| | | MCH | **31.3** pg | 27 - 32 |
| | | MCV | **88.2** fL | 83 -101 |
| | | Red Blood Cells (RBC) | **2.62** 10^12/L | 4.5 - 5.5 |
| | | HCT /PCV | **23.1** % | 40.0 - 50.0 |
| | | HB (Haemoglobin) | **8.2** g/dL | 13.0 - 17.0 |

## RADIOLOGY AND IMAGING INVESTIGATION(S) :

| Date | Test Name | Impression |
|---|---|---|
| 09-09-2026 | CARIOTID COLOR DOPPLER (ULTRA SOUND) | Normal Carotid Doppler study on both sides.<br>Normal vertebral arterial system on both sides |
| 09-09-2026 | EEG -BED SIDE | Normal awake EEG, |

## TREATMENT ON DISCHARGE

**R$_x$**

1. TAB **FIMOXYCLAV 1 TAB** ( AMOXICILLIN 875 MG + CLAVULANIC ACID 125 MG TAB )
   1+1+1 - [14 Days] - S.F 09/09/26 NOON ( Qty. 42 )
2. TAB **TREVOX 500 TAB** ( LEVOFLOXACIN 500 MG TAB )
   0+1+0 - [14 Days] - S.F 09/09/26 ( Qty. 14 )
3. TAB **TELMIPRES 40 TAB** ( TELMISARTAN 40 MG TAB )
   1+0+0 - [continue] - CHECK BP ( Qty. 0 )
4. TAB **NAPA EXTEND TAB** ( PARACETAMOL 665 MG TAB )
   1+1+1 - IF PAIN,FEVER ( Qty. 0 )
5. TAB **RIVOTRIL 0.5MG TAB** ( CLONAZEPAM 0.5 MG TAB )
   0+0+1 - [15 Days] - after meal ( Qty. 15 )
6. OINT **VIODIN 5% OINT** ( POVIDONE IODINE 5% OINT )

D/c Amlodipine - 10
0+0+1

Cont: Amenca Nalz

Add:
Methylphenidate
Modafinil

app Nalm
on Monday

10-04-2?

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## TREATMENT ON DISCHARGE

1+1+1 - Head bruise area ( Qty. 0 )

7. TAB **OMIDON 10 TAB** ( DOMPERIDONE 10 MG TAB )
   1+1+1 - [3 Month] - Before Meal ( Qty. 270 )
8. SUSP **ULFATE SUSP** ( SUCRALPHATE 1 GM/5ML SUSP )
   2 TSF X 6 HOURLY - [2 Month] - 30 MINS BEFORE MEAL ( Qty. 1 )
9. CAP **TRAXYL 500 CAP** ( TRANEXAMIC ACID 500 MG CAP )
   1+1+1 - ORAL - [3 Days] - After Meal ( Qty. 9 )
10. TAB **NEXUM MUPS 20 TAB** ( ESOMEPRAZOL 20 MG MUPS TAB )
    1+0+1 - ORAL - Before Meal ( Qty. 0 )
11. POW **ISABGUL PLUS (20S) SACHET** ( ISPAGHULA HASK 3 GM + SONAPATA 1 GM / 4 GM SACHET )
    1+0+1 - [CONTINUE] ( Qty. 0 )
12. TAB **FOLISON TAB** ( FOLIC ACID 5 MG TAB )
    1+0+1 - ORAL - [2 Month] - After Meal ( Qty. 120 )

## ADVICE

- FOLLOW UP WITH DR. ATM. MOHIUDDIN CHOWDHURY[GASTROENTEROLOGY] AFTER 15 DAYS.
- Please take medicine regularly
- In case of any emergency, please attend ER. For any routine queries call: +88 09612 247 247.

## DIET

- Normal

**Prepared By(Electronic Signature)**
**DR. SHABNUR MOSTHARI EMA**

**Consultant By(Electronic Signature)**
**DR. MOHAMMAD MAJIBUL HAQUE**